READING SUB-TEST – QUESTION PAPER: PART B

CANDIDATE NUMBER:

LAST NAME:

FIRST NAME:

OTHER NAMES:

PROFESSION:

VENUE:

TEST DATE:

CANDIDATE SIGNATURE _____

Passport Photo

INSTRUCTIONS TO CANDIDATES:

DO NOT open this **Question Paper** until you are told to do so.

One mark will be granted for each correct answer.

Answer **ALL** questions. Marks are **NOT** deducted for incorrect answers.

At the end of this test, hand in this **Question Paper**.

DO NOT remove OET material from the test room.

HOW TO ANSWER THE QUESTIONS:

Mark your answers on this **Question Paper** by filling in the circle using a 2B pencil.

Example:

Ⓐ
Ⓑ
Ⓒ

www.e2language.com

Part B

In this part of the test, there are six short extracts relating to the work of health professionals. For **questions 1-6**, choose answer (**A**, **B** or **C**) which you think fits best according to the text.

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1. The code of conduct applies to

Ⓐ doctors friending patients on Facebook.

Ⓑ privacy settings when using social media.

Ⓒ electronic and face to face communication.

Professional obligations
The Code of conduct contains guidance about the required standards of professional behaviour, which apply to registered health practitioners whether they are interacting in person or online. The Code of conduct also articulates standards of professional conduct in relation to privacy and confidentiality of patient information, including when using social media. For example, posting unauthorised photographs of patients in any medium is a breach of the patient's privacy and confidentiality, including on a personal Facebook site or group, even if the privacy settings are set at the highest setting (such as for a closed, 'invisible' group).

2. Why does dysphagia often require complex management?

Ⓐ Because it negatively influences the cardiac system.

Ⓑ Because it is difficult contrast complex and non-complex cases.

Ⓒ Because it seldom occurs without other symptoms.

6.1 General principles

Dysphagia management may be complex and is often multi-factorial in nature. The speech pathologist's understanding of human physiology is critical. The swallowing system works with the respiratory system. The respiratory system is in turn influenced by the cardiac system, and the cardiac system is affected by the renal system. Due to the physiological complexities of the human body, few clients present with dysphagia in isolation.

6.2 Complex vs. non-complex cases

Broadly the differentiation between complex and non-complex cases relates to an appreciation of client safety and reduction in risk of harm. All clinicians, including new graduates, should have sufficient skills to appropriately assess and manage non-complex cases. Where a complex client presents, the skills of an advanced clinician are required. Supervision and mentoring should be sought for newly graduated clinicians or those with insufficient experience to manage complex cases.

3. The main point of the extract is

Ⓐ how to find documents about infection control in Australia.

Ⓑ that dental practices must have a guide for infection control.

Ⓒ that dental infection control protocols must be updated.

1 Documentation
1.1 Every place where dental care is provided must have the following documents in either hard copy or electronic form (the latter includes guaranteed Internet access). Every working dental practitioner and all staff must have access to: a). a manual setting out the infection control protocols and procedures used in that practice, which is based on the documents listed at sections 1.1(b), (c) and (d) of these guidelines and with reference to the concepts in current practice noted in the documents listed under References in these guidelines b). The current Australian Dental Association Guidelines for Infection Control (available at: http://www.ada.org.au)

4. Negative effects from prescription drugs are often

Ⓐ avoidable in young people.

Ⓑ unpredictable in the elderly.

Ⓒ caused by miscommunication.

Reasons for Drug-Related Problems: Manual for Geriatrics Specialists

Adverse drug effects can occur in any patient, but certain characteristics of the elderly make them more susceptible. For example, the elderly often take many drugs (polypharmacy) and have age-related changes in pharmacodynamics and pharmacokinetics; both increase the risk of adverse effects.

At any age, adverse drug effects may occur when drugs are prescribed and taken appropriately; e.g., new-onset allergic reactions are not predictable or preventable. However, adverse effects are thought to be preventable in almost 90% of cases in the elderly (compared with only 24% in younger patients). Certain drug classes are commonly involved: antipsychotics, antidepressants, and sedative-hypnotics.

In the elderly, a number of common reasons for adverse drug effects, ineffectiveness, or both are preventable. Many of these reasons involve inadequate communication with patients or between health care practitioners (particularly during health care transitions).

5. The guideline tries to use terminology that

Ⓐ presents value-free information about different social groups.

Ⓑ distinguishes disadvantaged groups from the traditional majority.

Ⓒ clarifies the proportion of each race, gender and culture.

Terminology

Terminology in this guideline is a difficult issue since the choice of terminology used to distinguish groups of persons can be personal and contentious, especially when the groups represent differences in race, gender, sexual orientation, culture or other characteristics. Throughout the development of this guideline the panel endeavoured to maintain neutral and non-judgmental terminology wherever possible. Terms such as “minority”, “visible minority”, “non-visible minority” and “language minority” are used in some areas; when doing so the panel refers solely to their proportionate numbers within the larger population and infers no value on the term to imply less importance or less power. In some of the recommendations the term “under-represented groups” is used, again, to refer solely to the disproportionate representation of some citizens in those settings in comparison to the traditional majority.

6. What is the purpose of this extract?

Ⓐ To illustrate situations where patients may find it difficult to give negative feedback.

Ⓑ To argue that hospital brochures should be provided in many languages.

Ⓒ To provide guidance to people who are victims of discrimination.

Special needs

Special measures may be needed to ensure everyone in your client base is aware of your consumer feedback policy and is comfortable with raising their concerns. For example, should you provide brochures in a language other than English?

Some people are less likely to complain for cultural reasons. For example, some Aboriginal people may be culturally less inclined to complain, particularly to non-Aboriginal people. People with certain conditions such as hepatitis C or a mental illness, may have concerns about discrimination that will make them less likely to speak up if they are not satisfied or if something is wrong.